



TO SEND OUT

Vytalix — Business plan

Vytalix company-build documentation

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR

The Founder, Vytalix

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VERSION

1.0 · Confidential draft

For a bank, a grant body or an accountant. Send it when someone needs the whole company in one document.

This plan is honest about a company that has not started trading. Nothing below claims a customer, a partner, an approval or a pound of revenue that exists today. Every number is an ESTIMATE (our best guess, not a fact) built from stated assumptions.

1. The company

Vytalix is a British health-technology company, founder-led and about to be registered in England and Wales. It has four parts. Advisory and Consulting sell senior help to health organisations and health-technology firms. Health Solutions is building MaternaLink, a maternity coordination product. E-Learning will sell short practical courses. The founder holds all the shares at the start. There are no employees, no outside money, no revenue and no product in use. The plan below is how that changes.

2. The problem

Maternity care fails most often in the gaps between people. A woman sees a different midwife each visit. A concern raised on Tuesday is not on the screen on Friday. A handover between a community team and a hospital gets lost.

In the UK, public inquiries into maternity services keep finding the same failures: nobody listened, nobody escalated in time, the handover was lost. The national maternal death rate was 12.82 per 100,000 maternities in the 2021–23 reporting period, and Black women were more than twice as likely to die as White women.

In Nigeria, more women die in pregnancy and childbirth than in any other country. The reasons are delays in deciding to seek care, delays in reaching a facility and delays in receiving care once there. Phones are common. Coordination is not.

Nobody sells a tool that sits in that gap in both places to the same standard. That is the opening.

3. What we sell

Three things, priced and packaged today:

What	Price (estimate)
Advisory days	£1,200–£1,800 a day
Advisory retainers	£4,000 a month, three months minimum
Part-time strategy director	£5,500–£8,000 a month, six months
Fixed-price consulting projects (nine packages)	£9,500–£28,000 each
Training workshops	£4,000 a day
Online courses and team licences (from month nine)	£49–£299 a person; £2,500–£25,000 a team licence

MaternaLink is not for sale. It is in development and will not be sold until it passes the gates in section 7.

4. Who buys it

Four kinds of buyer, in the order we can realistically reach them:

1. **Health-technology companies.** They need help entering the UK market, passing NHS checks and raising money. They decide fast and pay on time. First revenue comes from here.
2. **Charities, foundations and non-profits running health programmes.** They buy business cases, evaluations and market scans.
3. **Private health providers.** Shorter buying cycles than the NHS, and real budgets.
4. **NHS trusts and local health boards.** The slowest to buy, at three to nine months, usually through a framework or as a sub-contract to a larger firm. Worth the wait for credibility.

Governments in Nigeria and Kenya are a later market for the product, not a first market for services.

5. The market

England has about 120 NHS trusts providing maternity care, covering roughly 568,000 births a year. If a maternity coordination product sold at £30,000 to £120,000 a trust a year, the whole English market is worth between £3.6m and £14m a year. That is the ceiling, not the target. Five to ten trusts over five years is the realistic ambition.

The services market is far larger and does not need a product. UK health consultancy is measured in hundreds of millions a year and is dominated by large firms. A boutique needs only a handful of clients a year to be viable.

In Africa, the money for maternal digital programmes comes mostly from donors, not state budgets. Prices there are low: the best-known programme in Kenya reaches over 2.8 million women at under one US dollar per mother. Any price we charge must buy something that plain messaging does not.

6. Competitors

Who	What they have	What we would do differently
Large consultancies (Deloitte, PA Consulting, Carnall Farrar)	Scale, references, framework places	Fixed price, senior person does the work, weeks not quarters. We would also sub-contract to them
UK maternity record vendors (System C BadgerNet, K2 Athena, Magentus Euroking)	The hospital record itself, in hundreds of sites	We are not a records system and never will be. We sit beside the record
US maternity technology firms (Maven, Babyscripts, Bloomlife)	Hundreds of millions of dollars raised	They do not sell into the NHS or into African public programmes
African programmes (mDoc in Nigeria, Jacaranda in Kenya)	Live programmes, donor funding, real scale	They set the price expectation. In one Nigerian state, mDoc is already the incumbent, which is why we will not compete there head-on
Free apps (Baby Buddy)	Free, trusted, widely recommended	Free apps do not do facility-side coordination or handover

Honest position: no competitor sits exactly where we intend to. Every neighbouring position is held by someone with more money.

7. The product

MaternaLink is a maternity coordination platform, in development. Version one is deliberately narrow: appointments and contact, two-way messaging that escalates to a real person, education a clinician has approved, and simple monitoring a midwife acts on. It does not diagnose and it does not decide anything for a clinician.

An earlier concept, an artificial-intelligence score predicting complications, is now a research project for later. It needs data partners, ethics approval and a formal route through the MHRA (the UK regulator for medicines and medical devices) before anyone can sell it.

Four gates must be passed before a pound of product revenue:

1. Written ownership of the product and its code, agreed with the collaborator who built the prototype.
2. A written statement of what the product is for, and a clinical safety approach signed off by a qualified safety officer.
3. A data protection assessment and the NHS digital checks (DTAC, the checklist a digital product must pass before a hospital can buy it).
4. One site willing to run it and be evaluated.

Today: a prototype exists and is under review. No hospital, clinic or government uses it. It is not NHS-approved, not regulator-cleared, and carries no CE or UKCA mark.

8. How we will grow

Year one is services only. The founder sells directly: 40 contacts, 20 conversations and two proposals a week. Three signed engagements in the first 90 days. Two retainers by month six. Published research on maternity coordination in the UK and Nigeria to earn attention without advertising.

Year two adds courses, one unpaid or grant-funded design site for MaternaLink, and a first grant application. Year three is paid pilots of MaternaLink, if and only if the four gates are passed.

The order matters: services pay for the company, the company pays for the product, the product needs evidence before it needs salespeople.

9. How the company is set up

One company registered in England and Wales, with the four parts as internal divisions. Not a group. A group means several sets of accounts a one-person company cannot afford.

Registrations in the first month: Companies House, corporation tax, the ICO data protection fee, professional indemnity and public liability insurance, and a business bank account. An application for SEIS and EIS advance assurance (tax schemes that give people money back when they invest in a small British company) follows registration.

Governance: a board of the founder plus one independent director, and an advisory board of six with obstetric, midwifery, NHS digital, Nigerian health system, regulatory and health-technology experience. Nobody is appointed yet, and nobody will be named without their written consent. A separate company for the product will be created later, and before any real patient data is handled.

10. The money

All figures are scenario outputs from stated assumptions. There are no actuals, because there is no trading history.

Scenario	Year 1	Year 2	Year 3	Three-year total
Base	£166,000	£488,000	£820,000	£1,474,000

In the base case, Advisory and Consulting are 59% of three-year revenue and are profitable from month two. MaternaLink is 26% and earns nothing before month 13.

Money question	Base answer
Cash needed in the first 12 months	About £36,000
Cash needed over 36 months, before any outside money	About £1,058,000
Pre-seed sought	£400,000 in month four
Seed sought	£2,000,000 in month 20
Profitable within 36 months	No. The base case deliberately spends the seed on building the product
Cash left at month 36	About £1,417,000, roughly 23 months of running costs

The careful case is the one a bank should read first: £413,000 of revenue over three years, a £250,000 pre-seed, no second technical hire, and about two months of cash left at month 36. It survives. It does not build much of a product.

Break-even on services alone needs about £7,500 of revenue a month against fixed costs of about £4,500. That is five billable days or two retainers a month.

11. The risks

Risk	What it would do	What we do about it
The product ownership agreement is not signed	We could not truthfully say we own MaternaLink, and no investor would fund it	Written heads of terms in week one, signed deed by day 30, hard stop at day 60. If it fails, we rebuild version one from scratch and drop the old name
The company name is not clear to use	Wasted spend on brand and domain	Trademark and company-name checks before any money is spent on printing
Nobody pays £1,200–£1,800 a day for a founder with no client list	The whole cash plan fails	Test three price points in the first six proposals. If buyers push below £900, cut costs and reposition
The founder is the only person who can sell or deliver	Illness or distraction stops everything	Cap billable days at 40–50% of capacity. Build associate relationships before they are needed
African governments will not pay above a few pounds per woman	Africa becomes an evidence market, not a revenue market	Price the model at £3–£12 a woman a year and treat Nigeria as proof, not profit. The £300 a woman figure in a March 2026 proposal is withdrawn
Regulation catches the product	Delay of a year or more, and cost	Version one is designed to stay non-diagnostic. A regulatory opinion is obtained before any clinical claim
Grant applications fail	Slower product build	Assume they fail. Grants are upside in every scenario, never the plan

12. The next 90 days

By	What must be true
Day 3	The narrow, non-diagnostic scope of MaternaLink version one is adopted in writing, and the old predictive claims are withdrawn from every document

By	What must be true
Day 12	The company is registered, with a bank account, insurance and ICO registration under way
Day 30	The product ownership deed is signed. The first paid engagement is signed. The prototype review is finished
Day 60	The Nigeria decision is made: partner, move state, or stop for six months. Trademark filed. Two advisers signed up
Day 90	Three engagements signed, worth £15,000–£30,000. Research report published. A build-or-wait decision on the product is minuted

If the first 90 days deliver those five lines, the company is real. If they do not, the plan changes rather than the story.

Contact: [Founder name] · [email] · [phone] · [website] · [Legal company name], registered in England and Wales, company number [●].